This Patient Group Direction (PGD)is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction
for the administration of Varivax and Varilrix for vaccination against Chickenpox

Patient Group Direction, version 007, issued 30 September 2026. Supersedes Version 006, 14 September 2026.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

Summary of the governing guidance for varicella vaccination: Green Book chapter 34 (Varicella), the Varivax and Varilrix SmPCs, and the JCVI advice of November 2023 on a routine childhood varicella programme. NICE CKS 'Chickenpox' covers the illness, not vaccination.

Vaccine Overview

- The varicella (chickenpox) vaccine is a **live attenuated vaccine**.
- Two licensed brands in the UK: **VARIVAX®** and **VARILRIX®**.
- Both vaccines are given subcutaneously. The route row of each arm of this PGD governs: Varivax by subcutaneous injection; Varilrix by subcutaneous or intramuscular injection.

Indications for Vaccination

The varicella vaccine is recommended for:

1. **Susceptible healthcare workers** and those in contact with immunocompromised individuals (to prevent transmission).
2. **Susceptible close household contacts** of immunocompromised patients.
3. **Non-immune adults and adolescents** (13 years and older) with no history of chickenpox, especially women of childbearing age (with appropriate contraceptive advice).
4. **Susceptible children (over 12 months)** who are at high risk of severe chickenpox or pose a risk to vulnerable contacts.
5. **Outbreak control** in some institutional settings (e.g. schools, nurseries, hospitals).

Note: "Susceptible" = no reliable history of chickenpox or no evidence of immunity.

Dosing Schedule

- **Two doses** are required for full protection:
  - **First dose** at any time from 12 months of age.
  - **Second dose** given at least **4 to 8 weeks later**.
- If given as part of MMRV combination vaccine, different guidance may apply for under-2s (due to febrile seizure risk).

Contraindications

Do **not** administer the vaccine to:

- Individuals with a history of severe allergic reaction to a previous varicella vaccine dose or any component.
- Immunosuppressed individuals (unless under specialist advice).
- Pregnant women (and advise avoiding pregnancy for **at least 1 month** after vaccination).
- People with untreated active tuberculosis or current febrile illness requiring postponement.

Precautions

- Postpone in acute illness with fever.
- Avoid salicylates (e.g. aspirin) for 6 weeks after vaccination due to Reye's syndrome risk.
- Consider prior immunity testing in adults if uncertain.

Vaccine Effectiveness

- Two doses are about **90–95% effective** at preventing chickenpox.
- If breakthrough infection occurs, it is usually mild.

Adverse Effects

- Common: Local site reactions, mild fever, mild rash.
- Rare: Febrile seizures (especially with MMRV in <2 years), varicella-like rash, mild systemic symptoms.

Documentation and Reporting

- Record batch number, site of injection, and date.
- Report any adverse reactions via **Yellow Card Scheme**.

Patient Group Direction

for the supply/administration of

Varivax

for vaccination against

Chickenpox

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	• Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required ● All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service

Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Active immunisation against varicella (chickenpox) in individuals aged 12 months and over who are susceptible to the varicella-zoster virus and have no history of chickenpox infection or prior vaccination.
Inclusion criteria	- Individuals aged 12 months and older. - No history of chickenpox infection. - Has not completed a two-dose varicella course. Dose 2 may be given under this PGD where dose 1 was given elsewhere; record the date and brand of dose 1. - Informed consent given. - Suitable for vaccination in accordance with national immunisation guidelines.
Exclusion criteria	- History of chickenpox infection. - Completed two-dose varicella course. - Hypersensitivity to neomycin, gelatin, or any component of the vaccine. - Immunosuppression of any cause, including immunosuppressive therapy or high-dose systemic corticosteroids; blood dyscrasias, leukaemia, lymphoma or other malignancy of the blood or lymphatic system; family history of congenital or hereditary immunodeficiency unless immune competence has been demonstrated. Refer. - Pregnancy or planning pregnancy within one month. - Acute febrile illness or active untreated tuberculosis. - MMR or another live vaccine within the previous 4 weeks, unless given on the same day.
Cautions	- Avoid contact with high-risk individuals (e.g. immunosuppressed) for 4 to 6 weeks if a rash develops post-vaccination. - Pregnancy must be avoided for one month post-vaccination. - Postpone in the case of moderate or severe illness with fever. - Give on the same day as MMR or other live vaccines, or 4 weeks apart. - Immunoglobulin or blood products in the previous 3 months may reduce the response: where protection is needed vaccinate now and consider a further dose after 3 months (Green Book); record the reason.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Varivax
Legal category	POM
Storage	Store in a refrigerator (2°C–8°C). Do not freeze. Protect from light.
Route/method of administration	Subcutaneous injection, usually in the upper arm.

Dose and frequency	Two 0.5 mL doses. Varivax: second dose at least 4 weeks after the first (13 years and over: 4 to 8 weeks).
Quantity to be administered and/or supplied	0.5 mL per dose; course of two doses
Maximum or minimum treatment period	Two doses. The course is complete only after the second dose.
Adverse effects	Injection site reactions (pain, redness, swelling), fever, rash, irritability, or upper respiratory symptoms. Rarely, varicella-like rash or febrile convulsions. Refer to SmPC.

Yellow card reporting	Report any suspected adverse effects via https://yellowcard.mhra.gov.uk and notify the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of administration, dose, form, route and anatomical site, quantity administered, batch number and expiry date ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and the actions taken, and that the vaccine was administered under this PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	Attend for the second dose on the date given. Seek medical advice for a severe or persistent injection site reaction, a fever that does not settle within 48 hours, a widespread vesicular rash, or any sign of an allergic reaction; call 999 for difficulty breathing, swelling of the face or throat, or collapse. Avoid salicylates (aspirin) for 6 weeks after each dose. While any vaccine-related rash is present, avoid close contact with pregnant women, newborn babies and people with weakened immunity.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mg2
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources
- UKHSA, Immunisation Against Infectious Disease (the Green Book), chapter 34 Varicella; JCVI statement on a childhood varicella vaccination programme, November 2023; Varivax and Varilrix Summaries of Product Characteristics, current versions
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines BNF (British National Formulary) | NICE
- Summary of Product Characteristics

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date		Expiry date	
30/9/26		31/7/27	

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or vaccine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date

Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history to version 001

Later versions are recorded in the Version and change record at the end of this document.

Version number	Change details	Date
001	Development & issue of new PGD	25th June 2025

This Patient Group Direction (PGD)is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

for the supply/administration of

Varilrix

for vaccination against

Chickenpox

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	• Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required ● All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service

Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Active immunisation against varicella (chickenpox) in individuals aged 12 months and over who are susceptible to the varicella-zoster virus and have no history of chickenpox infection or prior vaccination.
Inclusion criteria	- Individuals aged 12 months and older. - No history of chickenpox infection. - Has not completed a two-dose varicella course. Dose 2 may be given under this PGD where dose 1 was given elsewhere; record the date and brand of dose 1. - Informed consent given. - Suitable for vaccination in accordance with national immunisation guidelines.
Exclusion criteria	- History of chickenpox infection. - Completed two-dose varicella course. - Hypersensitivity to neomycin, gelatin, or any component of the vaccine. - Immunosuppression of any cause, including immunosuppressive therapy or high-dose systemic corticosteroids; blood dyscrasias, leukaemia, lymphoma or other malignancy of the blood or lymphatic system; family history of congenital or hereditary immunodeficiency unless immune competence has been demonstrated. Refer. - Pregnancy or planning pregnancy within one month. - Acute febrile illness or active untreated tuberculosis. - MMR or another live vaccine within the previous 4 weeks, unless given on the same day.
Cautions	- Avoid contact with high-risk individuals (e.g. immunosuppressed) for 4 to 6 weeks if a rash develops post-vaccination. - Pregnancy must be avoided for one month post-vaccination. - Postpone in the case of moderate or severe illness with fever. - Give on the same day as MMR or other live vaccines, or 4 weeks apart. - Immunoglobulin or blood products in the previous 3 months may reduce the response: where protection is needed vaccinate now and consider a further dose after 3 months (Green Book); record the reason.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Varilrix
Legal category	POM
Storage	Store in a refrigerator (2°C–8°C). Do not freeze. Protect from light.
Route/method of administration	Subcutaneous or intramuscular injection.

Dose and frequency	Two 0.5 mL doses. Varilrix: second dose preferably 6 weeks after the first, and never less than 4 weeks (SmPC). Record the reason where the interval is between 4 and 6 weeks.
Quantity to be administered and/or supplied	0.5 mL per dose; course of two doses
Maximum or minimum treatment period	Two doses. The course is complete only after the second dose.
Adverse effects	Injection site reactions, fever, rash, irritability. Rare: febrile seizures, varicella-like rash. Refer to SmPC for full adverse effect profile.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as necessary.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of administration, dose, form, route and anatomical site, quantity administered, batch number and expiry date ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and the actions taken, and that the vaccine was administered under this PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

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Name of healthcare professional	Job title	Registration number	Signature
Valid from date		Expiry date	
30/9/26		31/7/27	

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Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date

Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history to version 001

Later versions are recorded in the Version and change record at the end of this document.

Version number	Change details	Date
001	Development & issue of new PGD	25th June 2025

Vaccine safety requirements

The requirements on the following pages form part of this Patient Group Direction and must be met before any vaccine is administered under it.

Anaphylaxis: what must be in place before you vaccinate

ADRENALINE (EPINEPHRINE) 1 IN 1,000 INJECTION must be immediately available in the room where vaccination takes place, in date, together with a telephone.

A written anaphylaxis protocol consistent with current Resuscitation Council UK guidance must be available, and every person administering must be trained in the recognition and immediate management of anaphylaxis and hold current basic life support training.

Be able to distinguish a faint and a panic attack from anaphylaxis. Green Book chapter 8 sets out the clinical features of each. A faint after an injection is common; anaphylaxis is very rare.

Report any suspected anaphylaxis via the Yellow Card Scheme even where the diagnosis is uncertain.

Observation after vaccination

OBSERVE EVERY PATIENT FOR 15 MINUTES AFTER VACCINATION, SEATED.

Anxiety-related reactions including vasovagal syncope, hyperventilation and transient visual disturbance or paraesthesia can occur before or after any injection, and are commonest in adolescents. They are a response to the needle, not to the vaccine.

Have procedures in place to prevent injury from a faint.

Record that the observation period was completed.

Cold chain and excursions

Store at +2C to +8C in the original packaging to protect from light. Do not freeze, and discard any vaccine that has been frozen.

COLD CHAIN EXCURSION: any stock exposed to conditions outside +2C to +8C must be QUARANTINED and risk assessed in accordance with UKHSA Vaccine Incident Guidance before any further use.

Do not administer excursion stock until that assessment is complete and has been recorded.

Where the product SPC gives specific stability data for a temporary excursion, that data governs; otherwise quarantine and assess.

Disposal

Dispose of used syringes, needles, vials and any unused or reconstituted vaccine in a UN-approved puncture-resistant sharps container.

Follow local authority requirements and Health Technical Memorandum 07-01, Safe management of healthcare waste.

Never re-sheath a needle.

Records: traceability

RECORD THE VACCINE NAME AND BATCH NUMBER for every dose administered. This is a requirement for any biological product and it is what makes a recall actionable.

Record also the expiry date, the dose, the route, the anatomical site, and where more than one vaccine was given at the same visit, the site of each.

Record the name and registration number of the person administering, and that the vaccine was given under this PGD.

Consent in children and young people

Where the individual is UNDER 16, valid consent must come from a person with PARENTAL RESPONSIBILITY, or from the young person where you assess them as GILLICK COMPETENT.

Record which of the two applied. Where consent was given by a person with parental responsibility, record their name and relationship to the patient. Where the young person consented on the basis of Gillick competence, record the basis of that assessment.

A parent accompanying a child does not automatically hold parental responsibility. Ask.

Users must be competent in assessing consent in children and young people before working under this PGD.

Version and change record

Version: v007

Issued: 30 September 2026

Supersedes: Version 006, 14 September 2026

Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

Previous version records

Version: v006

Issued: 14 September 2026

Supersedes: Version 005, 11 September 2026

Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.

Previous version records

Version: v005

Issued: 11 September 2026

Supersedes: Version 004, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

The product name Varilrix was misspelt 'Varilirix' in the title, on the Varilrix cover and in its name of medicine row; corrected. The Varilrix cover no longer says 'for the treatment of Chickenpox'.

The guidance summary's statement that intramuscular administration is acceptable for the vaccine generally now defers to the route row of each arm (Varivax subcutaneous; Varilrix subcutaneous or intramuscular).

The maximum treatment period rows in both arms now say two doses, matching the dose rows and the version 003 change record; the previous 'one or two doses' wording contradicted them.

The Varilrix interval now reads as the SmPC has it: preferably 6 weeks, never less than 4, with the reason recorded for an interval between 4 and 6 weeks.

The records rows have their punctuation restored and ask for the site, batch number and expiry date; the follow-up advice rows, which carried the treatment template about 3 to 4 weeks, now give vaccination follow-up advice including salicylate avoidance and contact precautions for a vaccine rash.

Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.

Previous version records

Version: v004

Issued: 11 September 2026

Supersedes: Version 003, 10 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

The guidance summary and references are attributed to the Green Book, JCVI and the SmPCs rather than to a NICE CKS vaccination topic that does not exist.

Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Previous version records

Version: v002

Issued: 9 September 2026

Supersedes: the previous signed version of this document.

Change: the vaccine safety requirements above were added following an estate-wide sweep of every vaccination PGD, which found that the signed documents were missing, between them, any requirement for adrenaline to be available, any anaphylaxis provision, a stated observation period, a cold chain excursion procedure, a sharps disposal provision, a batch number requirement, and any wording on consent in children and young people. The specific items added to THIS document are: Anaphylaxis: what must be in place before you vaccinate, Observation after vaccination, Cold chain and excursions, Disposal, Records: traceability, Consent in children and young people.

Authorised by Nitin Shori, Medical Director (GMC 6047293) and Chris Pilkington, Head Pharmacist (GPhC 2046322), on 9 September 2026. Signatures applied digitally on their joint instruction, which is the established process for Get Real Health PGDs.

Version: v003

Issued: 10 September 2026

Supersedes: Version 002, 9 September 2026

Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Validity: this version is valid from 10 September 2026 and expires on 31 July 2027. The signed master previously stated no period of validity, or an expiry of 31 October 2026; those are superseded. No period of validity was stated in either arm.

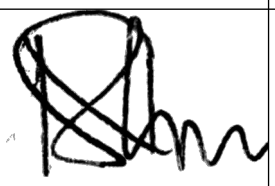
Exclusion criteria, both arms: 'Immunocompromised individuals unless under specialist advice' replaced by the SmPC contraindications: immunosuppression of any cause, blood dyscrasias, leukaemia, lymphoma or other malignancy of the blood or lymphatic system, and family history of congenital or hereditary immunodeficiency unless immune competence is demonstrated.

Dose: both SmPCs and the Green Book require two doses. The previous text made the second dose optional. Now two 0.5 mL doses: Varivax at least 4 weeks apart (13 years and over: 4 to 8 weeks); Varilrix at least 6 weeks apart and never less than 4. Inclusion no longer excludes a patient attending for dose 2.

Cautions: MMR and other live vaccines must be given on the same day or 4 weeks apart; blood products or immunoglobulin in the previous 3 months handled per the Green Book. 'Recently' was undefined.

Signed on behalf of Get Real Health

Version v007, 30 September 2026.

Name	***Qualification***	***Signature***	***Date***
Nitin Shori	Medical Director GMC: 6047293		30 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		30 September 2026

Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.